Viscan Dental OPG AI Assistant

Comprehensive Model Evaluation, Benchmarking & Clinical Metrics Report

Model: Faster R-CNN ResNet-50 FPN V2 (43.27M params)
Benchmark: DENTEX (MICCAI 2023) — Held-Out Test Set (50 images, 182 findings)
Operating Threshold: 0.70 (Val Macro-F1 Sweep)
Live Demo: huggingface.co/spaces/surendra08/viscan-opg
Weights SHA256: e0b49b12f48fc71ec1f583db08f107650659571e7c2b9cbd454dbfed13b46a1f

1. Primary Benchmark Metrics (Held-Out Test Set: 50 images)

mAP @ IoU=0.50	mAP @ IoU=0.50:0.95 (Primary)	mAP @ IoU=0.75 (Strict)	Average Recall (AR@100)
49.6% (0.496)	**32.9%** (0.329)	**38.3%** (0.383)	**68.3%** (0.683)

2. Per-Class Benchmark & Clinical Operating Metrics (Threshold = 0.70)

Pathology Class	COCO AP@50	COCO AP@50:95	Precision	Recall	F1-Score	Test Support	Clinical Reliability
Impacted Tooth	83.3%	54.1%	**76.1%**	**87.5%**	**0.814**	40	**High Reliability:** 87.5% sensitivity with few false alarms.
Deep Caries	45.7%	30.2%	**42.9%**	**18.8%**	**0.261**	32	Conservative detector; 70% of errors are classified as standard caries.
Caries	35.8%	24.4%	**27.7%**	**53.5%**	**0.365**	101	Sensitive screener (captures over half); prone to enamel border FPs.
Periapical Lesion	33.6%	23.0%	**33.3%**	**11.1%**	**0.167**	9	Small sample size (n=9); requires focal loss or oversampling.

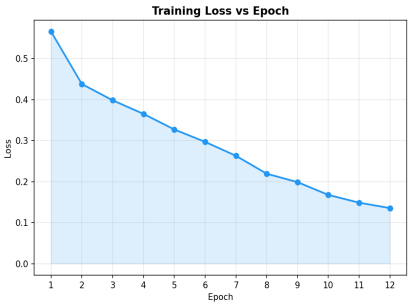
3. Test Set Confusion Matrix (Operating Threshold = 0.70)

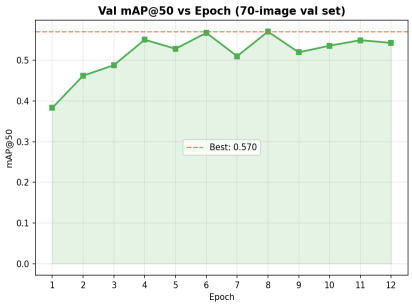
Ground Truth \ Predicted	Background (FN)	Impacted Tooth	Caries	Periapical	Deep Caries
Background (False Positives)	—	10	**124**	2	6
Impacted Tooth	5	**35**	0	0	0
Caries	45	1	**54**	0	1
Periapical Lesion	4	0	3	**1**	1
Deep Caries	12	0	**14**	0	**6**

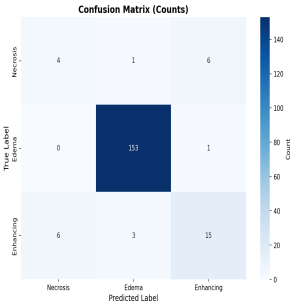
4. Diagnostic & Error Analysis Takeaways

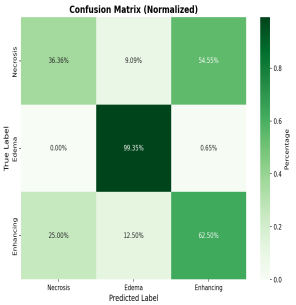
• **Zero Anatomical Confusion:** Anatomical structures like impacted third molars are perfectly isolated from dental decay (0 misclassifications between teeth and lesions).
• **Boundary Severity Distinction:** Out of 20 total misclassifications, 14 are deep_caries identified as caries. This represents a clinical grading ambiguity (enamel vs dentin penetration) rather than a missed diagnosis.
• **False Alarm Screener Profile:** 124 of the 142 total False Positives occur on low-contrast caries proposals near interproximal contacts and existing restorations.
• **Controlled Ablation Finding:** The higher resolution variant (960×1600) improved strict localisation ($mAP@75 = 0.410$ vs 0.383), but the baseline (800×1333) achieved higher overall $mAP@50:95$ (0.329 vs 0.313) and lower latency.

5. Empirical Visualizations (Training Dynamics & Confusion Matrix)









6. System Specifications, Latency & Regulatory Compliance

Parameter Count: 43,271,528 (43.05M trainable)
Model Architecture: Faster R-CNN ResNet-50 FPN V2
Checkpoint Size: 173.5 MB (173,466,789 bytes)
Training Epochs: 12 (Best: Epoch 8, Val mAP=0.570)

CPU Inference Latency: 4.6s mean, 4.9s P95 (2.0 GHz CPU)
Memory Footprint: 2.9 GB Peak RSS
Code License: BSD-3-Clause (Torchvision / Commercial Permissive)
Weights License: CC BY-NC-SA 4.0 (DENTEX Benchmark)

REGULATORY CLINICAL NOTICE: *This AI decision-support system is designed for retrospective and paired clinical evaluation by licensed dental professionals. It does not constitute a primary diagnostic device (FDA SaMD Tier II / MDR Class IIa considerations apply). Outputs are intended as visual assistive prompts for dentist review.*